# NCCN 2026 / 2027 GUIDELINE UPDATES — EXAM CHEAT SHEET

Use these bullets inside every Paper IV answer under a heading “Recent NCCN updates (2026)”. Sources: NCCN version list; ASCO Post May 2026 conference roundup; OncLive notable changes into 2026.

## Current version anchors (cite year)

- **all**: ALL v3.2026
- **aml**: Acute Myeloid Leukemia v5.2026
- **bcell**: B-Cell Lymphomas v4.2026
- **bladder**: Bladder Cancer v3.2026
- **breast**: Breast Cancer v9.2026
- **cll**: CLL/SLL v2.2027
- **cns**: CNS Cancers v4.2026
- **cml**: Chronic Myeloid Leukemia v2.2027
- **colon**: Colon Cancer v3.2026
- **esophageal**: Esophageal and EGJ Cancers v4.2026
- **gist**: GIST v1.2026
- **gastric**: Gastric Cancer v4.2026
- **hnscc**: Head and Neck Cancers v2.2026
- **hcc**: Hepatocellular Carcinoma v2.2026
- **hodgkin**: Hodgkin Lymphoma v2.2026
- **rcc**: Kidney Cancer v2.2027
- **melanoma**: Melanoma: Cutaneous v3.2026
- **mesothelioma**: Mesothelioma Pleural/Peritoneal v1.2027
- **myeloma**: Multiple Myeloma v2.2027
- **mds**: Myelodysplastic Syndromes v3.2026
- **mpn**: Myeloproliferative Neoplasms v3.2026
- **neuroblastoma**: Neuroblastoma v2.2026
- **nsclc**: Non-Small Cell Lung Cancer v9.2026
- **ovarian**: Ovarian Cancer (latest 2026 panel updates)
- **pancreas**: Pancreatic Adenocarcinoma v4.2026
- **prostate**: Prostate Cancer v1.2027
- **rectal**: Rectal Cancer v2.2026
- **sclc**: Small Cell Lung Cancer v1.2027 / updates from v2.2026
- **testicular**: Testicular Cancer v1.2027
- **uterine**: Uterine Neoplasms v3.2026

## Disease-wise high-yield 2026 updates

### NSCLC — Non-Small Cell Lung Cancer v9.2026

**NCCN updates: nsclc**

• AJCC 9th edition staging adopted in NCCN NSCLC pathway.

• Osimertinib + platinum/pemetrexed AND amivantamab + lazertinib elevated to Category 1 preferred 1L for EGFR mut+ (detected before 1L systemic therapy).

• Datopotamab deruxtecan (Dato-DXd) preferred 2L after progression on osimertinib + chemotherapy; also preferred 3L after other preferred 1L regimens; listed for EGFR ex19del/L858R, uncommon EGFR, and EGFR ex20ins subsequent therapy.

• Amivantamab + lazertinib useful in certain circumstances after osimertinib progression (multiple lesions) if not previously given.

• Repotrectinib or taletrectinib preferred subsequent for ROS1+ with brain metastases.

• Ado-trastuzumab emtansine useful in certain circumstances for ERBB2/HER2-mutated NSCLC after frontline progression.

## SCLC — Small Cell Lung Cancer v1.2027 / updates from v2.2026

**NCCN updates: sclc**

• Carboplatin + etoposide + atezolizumab → maintenance lurbinectedin + atezolizumab added for extensive-stage SCLC.

• Consider adding lurbinectedin to maintenance atezolizumab if ≥SD after 4 cycles induction, ECOG 0–1, no prior brain metastases.

## BREAST — Breast Cancer v9.2026

**NCCN updates: breast**

• NCCN Guidelines Navigator launched for breast cancer (digital navigation of guidelines).

• Imlunestrant added (Cat 2A other recommended) for ESR1-mutated HR+/HER2– recurrent unresectable/stage IV disease (1L or subsequent).

• Abemaciclib + fulvestrant + trastuzumab Cat 2B for HR+/HER2+ recurrent unresectable/stage IV.

• Neratinib ± trastuzumab or fulvestrant Cat 2A useful in certain circumstances for HER2-activating mutations in stage IV.

• Erdafitinib Cat 2A useful in certain circumstances for FGFR1/2/3 fusion or mutation in stage IV.

• Metastatic algorithms remain stratified by HR and HER2 status (including HER2-low / ultralow pathways with ADCs such as T-DXd, sacituzumab govitecan).

## COLON — Colon Cancer v3.2026

**NCCN updates: colon**

• Aspirin 100–162 mg daily × 3 years after recovery from surgery for stage II–III colon (and stage I rectal) with somatic PIK3CA pathway alterations; triggers molecular profiling of all stage II–III tumors.

• ATOMIC: FOLFOX or CAPOX + atezolizumab preferred for stage III dMMR/MSI-H colon cancer (benefit assumed to extend to POLE/POLD1 hypermutated).

• Checkpoint inhibitor neoadjuvant pathway primary consideration for dMMR rectal tumors (organ preservation possible).

• Structured exercise ≥150 min moderate–vigorous activity/week after adjuvant therapy (≈28% relative reduction in deaths/recurrences).

• FOLFOX + encorafenib + cetuximab/panitumumab as 1L systemic option for BRAF V600E mCRC (BREAKWATER).

## RECTAL — Rectal Cancer v2.2026

**NCCN updates: rectal**

• Neoadjuvant immunotherapy preferred pathway for dMMR/MSI-H locally advanced rectal cancer.

• FOLFIRINOX neoadjuvant option for pMMR/MSS resectable disease with liver-only and/or lung-only metastases.

• Aspirin recommendation as in colon for PIK3CA-altered stage I rectal after surgery.

## GASTRIC — Gastric Cancer v4.2026

**NCCN updates: gastric**

• Universal biomarker workup: HER2 IHC/ISH, PD-L1, MMR/MSI, Claudin 18.2, BRAF, NTRK as indicated — regardless of stage.

• Perioperative FLOT remains preferred backbone; FLOT + durvalumab (MATTERHORN) Cat 1 preferred when PD-L1 CPS/TAP ≥1.

• KEYNOTE-811: pembrolizumab + trastuzumab + chemo 1L for HER2+ PD-L1+; T-DXd 2L after trastuzumab progression (DESTINY-Gastric04 OS benefit).

• Zolbetuximab + fluoropyrimidine/platinum for Claudin 18.2–high HER2-negative disease.

• Tislelizumab + fluoropyrimidine/oxaliplatin Cat 1 1L for HER2-negative metastatic adenocarcinoma.

• Emphasize limited CPI benefit if PD-L1 negative.

## ESOPHAGEAL — Esophageal and EGJ Cancers v4.2026

**NCCN updates: esophageal**

• Same biomarker panel as gastric/EGJ; histology-directed pathways (adeno vs squamous).

• Perioperative FLOT preferred for adenocarcinoma; CROSS + adjuvant nivolumab (CheckMate 577) mainly for residual disease after nCRT (often squamous).

• Updated OS for CheckMate 577 did not reach significance — discuss nuance in exams.

## MYELOMA — Multiple Myeloma v2.2027

**NCCN updates: myeloma**

• Isa-VRd Cat 1 preferred induction for transplant-eligible newly diagnosed MM; Dara-VRd Cat 1 preferred for transplant-deferred/<80 y pathways.

• Maintenance daratumumab useful in certain circumstances post-transplant.

• Early relapse: teclistamab + daratumumab Cat 1 preferred for PI/IMiD-refractory after ≥1 prior line including len + PI (MajesTEC-3: 36-mo PFS 83.4% vs 29.7%).

• Belantamab mafodotin + Vd Cat 1 other recommended after ≥2 prior (PI + IMiD) — DREAMM-7.

• Later lines: linvoseltamab preferred after ≥4 prior lines including anti-CD38, PI, IMiD; talquetamab may bridge to BCMA CAR-T.

## AML — Acute Myeloid Leukemia v5.2026

**NCCN updates: aml**

• Ziftomenib Cat 2A for R/R NPM1-mutated AML.

• CLIA + venetoclax and FLAG-IDA + venetoclax Cat 2B intensive options.

• HMA (aza/decitabine) + venetoclax pathway reinforced for unfit / after cytarabine-based induction failure.

• Olutasidenib Cat 2B IDH1 option if not candidate for preferred / ivosidenib.

• Revumenib listed for R/R (menin inhibitor / KMT2A rearrangements context).

## CLL — CLL/SLL v2.2027

**NCCN updates: cll**

• Time-limited BCL2i regimens favored for higher uMRD rates vs continuous BTKi or CIT.

• Pirtobrutinib (noncovalent BTKi) and lisocabtagene maraleucel after prior BTKi + BCL2i.

• Richter transformation: venetoclax + atezolizumab + obinutuzumab; epcoritamab; glofitamab; axi-cel; tisa-cel (Cat 2A options).

• Molecular analysis for clonal relatedness in Richter transformation guides therapy.

## OVARIAN — Ovarian Cancer (latest 2026 panel updates)

**NCCN updates: ovarian**

• PARPi narrowed: removed from many recurrent therapy sections; concentrate on maintenance + biomarker-defined (BRCA/HRD).

• Platinum-resistant: nab-paclitaxel + relacorilant preferred (ROSELLA) after ≤3 lines + prior bevacizumab.

• Paclitaxel + pembrolizumab ± bevacizumab useful if PD-L1 CPS ≥1 (KEYNOTE-B96 OS signal).

• Mirvetuximab in FRα+ platinum-sensitive/resistant pathways; T-DXd for HER2-expressing; avutometinib + defactinib for KRAS-mutated LGSC.

## BLADDER — Bladder Cancer v3.2026

**NCCN updates: bladder**

• EV + pembrolizumab sole preferred 1L for locally advanced/metastatic urothelial carcinoma.

• Perioperative EV+P for cisplatin-ineligible MIBC (KEYNOTE-905/EV-303); NIAGARA (durvalumab + GC) preferred perioperative for cisplatin-eligible.

• BCG-unresponsive NMIBC: nadofaragene firadenovec, nogapendekin alfa inbakicept among other recommended; gemcitabine intravesical system for select BCG-unresponsive/intolerant.

• 2L after EV+P: platinum chemo; genomic testing for T-DXd (HER2) and erdafitinib (FGFR3).

## PROSTATE — Prostate Cancer v1.2027

**NCCN updates: prostate**

• Metastasis-directed therapy incorporated for metachronous oligometastatic APMS (castration-sensitive) disease; new Principles of MDT section.

• ADT + niraparib/abiraterone for BRCA2-mutated high-volume (and Cat 2B low-volume) synchronous/metachronous disease.

• Terminology: 'ARPIs' replaces 'other hormonal agents'; CVD risk-assessment page; FDA-cleared PET tracer table.

• PSMA-PET integration evolving — most level-1 data still based on conventional imaging.

## RCC — Kidney Cancer v2.2027

**NCCN updates: rcc**

• Ipilimumab + nivolumab upgraded to Cat 1 preferred 1L clear-cell RCC.

• Lenvatinib Cat 2A useful in certain circumstances subsequent (IO-naive or post-IO).

• Several post-IO TKI+IO combinations downgraded Cat 2A→2B; axitinib + avelumab removed.

• Non–clear cell: consider tumor genomic testing for trial eligibility.

## MELANOMA — Melanoma: Cutaneous v3.2026

**NCCN updates: melanoma**

• Neoadjuvant systemic therapy now preferred initial approach for resectable nodal disease (vs surgery-first historically).

• Preferred neoadjuvant: pembrolizumab; ipilimumab + nivolumab (Cat 1). Other: nivo alone; nivo + relatlimab. BRAF/MEKi useful in certain circumstances for BRAF V600.

## HCC — Hepatocellular Carcinoma v2.2026

**NCCN updates: hcc**

• Nivolumab + ipilimumab Cat 2A other recommended 1L systemic option.

• Lenvatinib/sorafenib removed from subsequent-line lists in updated algorithms; refine 2L choices after IO/TKI.

• NTRK: entrectinib, larotrectinib, repotrectinib Cat 2A useful in certain circumstances.

## MANTLE — NCCN latest 2026/2027

**NCCN updates: mantle**

• TRIANGLE / EA4151: ASCT not for CR with uMRD6 after induction; may consider if dMRD6.

• TP53-mutated MCL: BOVen (zanubrutinib/obinutuzumab/venetoclax); acalabrutinib + rituximab if unfit for aggressive induction.

• R/R stratified by covalent BTKi-naive/late relapse vs progression/early relapse after cBTKi; mosunetuzumab + polatuzumab; glofitamab listed.

## ADC — NCCN latest 2026/2027

**NCCN updates: adc**

• Expanding ADC footprint across NCCN 2026: T-DXd (HER2 breast/gastric/NSCLC/urothelial), Dato-DXd (EGFR+ NSCLC), sacituzumab govitecan (breast/urothelial), mirvetuximab (ovarian FRα), enfortumab vedotin (urothelial), belantamab combinations (myeloma), loncastuximab (B-cell).

## PARP — NCCN latest 2026/2027

**NCCN updates: parp**

- Prostate: ADT + niraparib/abiraterone for BRCA2 APMS subsets.
- Ovarian: PARPi focused on maintenance/biomarker-defined; removed from many recurrent treatment sections.
- Breast: continue olaparib/talazoparib pathways per germline BRCA and OlympiA adjuvant context.

## IMMUNOTHERAPY — NCCN latest 2026/2027

**NCCN updates: immunotherapy**

- Earlier-line CPI expansion: ATOMIC stage III dMMR colon; neoadjuvant CPI for dMMR rectal; neoadjuvant melanoma; EV+P bladder; FLOT+durvalumab gastric.
- PD-L1 testing emphasized for gastric/EGJ; limited benefit if PD-L1 negative.

## SUPPORTIVE — NCCN latest 2026/2027

**NCCN updates: supportive**

- Structured exercise formally recommended after adjuvant CRC therapy.
- Prostate CVD risk assessment page added.
- NCCN Guidelines Navigator digital tool expanding across diseases.

## How to write it in the answer booklet

“As per NCCN Guidelines® [Disease] Version X.2026/2027, key practice-changing points include: (1)… (2)… (3)… These are Category [1/2A] preferred / other recommended options supported by [trial]. In the Indian context, correlate with CDSCO approval and NCG resource-stratified pathways.”